

Therapeutic Formulation

Definition: Process for developing a hypothesis about, and a plan to address the causes, precipitants, and maintaining influences of a person's psychological, interpersonal, and behavioural problems in the context of that individual's culture and environment.

Formulation includes:

- Individual's behaviour: under- or over-expressed, normative or non normative, and adaptive or maladaptive.
- Interaction with others: Beliefs the person has about the intentions and wishes of others and what the responses are to those expectations.
- Individual's environment: cultural influences, social roles and conflicts.
- Potential influence of the physical environment on functioning: safety of the neighbourhood, socioeconomic factors, and education and work opportunities.
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Goals

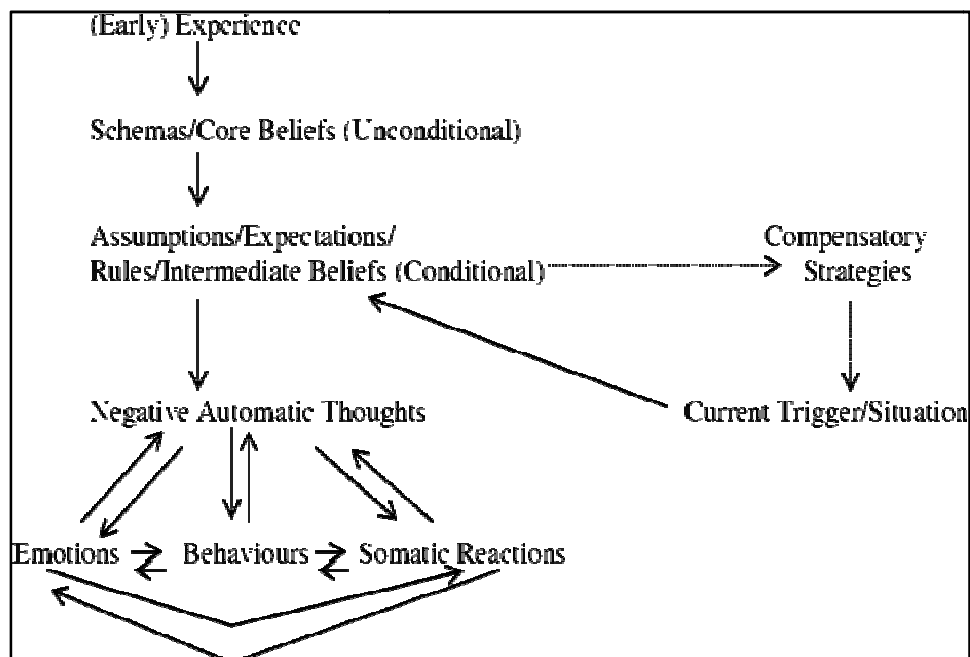
- Guides treatment by helping the therapist stay on track from one session to the next, monitor progress, and be alert when a change of direction is indicated.
- Increases treatment efficiency as the therapist can develop a time-effective, evidence-based plan from the beginning to the end of treatment.
- Tailors treatment to the specific
- circumstances a client is facing, considering the problems and range of diagnoses being addressed, and also the context of treatment.
- A formulation helps therapist understand the patient better, thus enhancing therapist empathy.

Functional Benefits

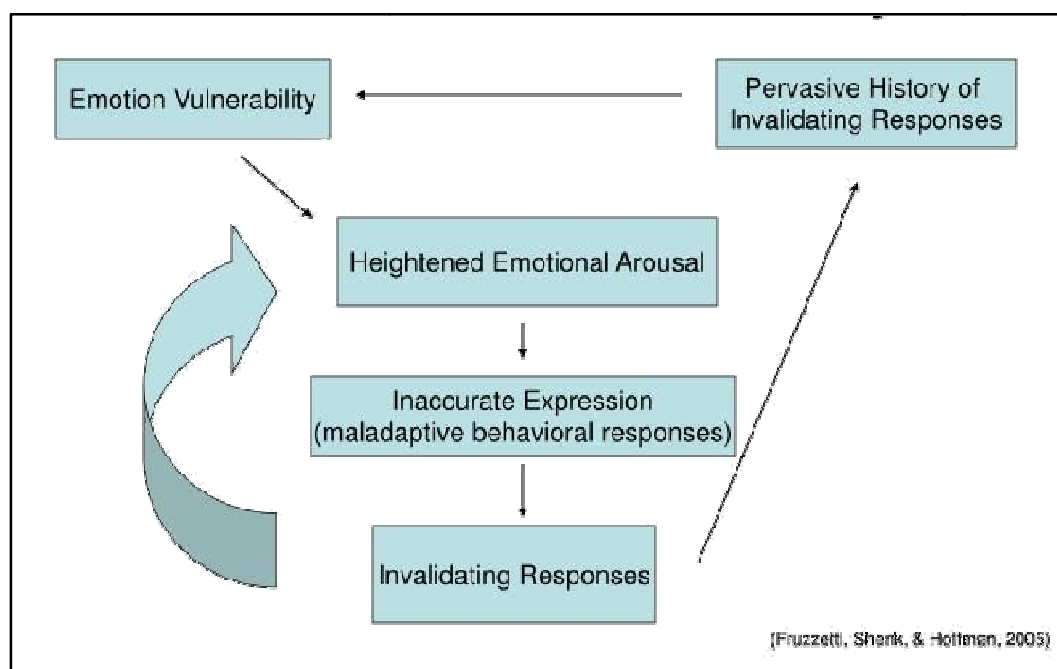
- Minimize bias & misplaced effort
- Integrate multiple perspectives on case
- Logically and sequentially "build" the case
- Develop a working model of the client to guide intervention & identify leverage points
- Ensure essential information is available and identify missing/needed information.
- Provide structure for generating hypotheses
- Ensure that all staff are unified in treatment focus
- Identify treatment amenability, focus, priorities, and approach treatment in a systematic way
- Help therapist anticipate problems that can arise
- Communicate the case and treatment clearly to others
- Shared with the client, it can strengthen therapeutic alliance, motivation & compliance.

Models of Case Formulation

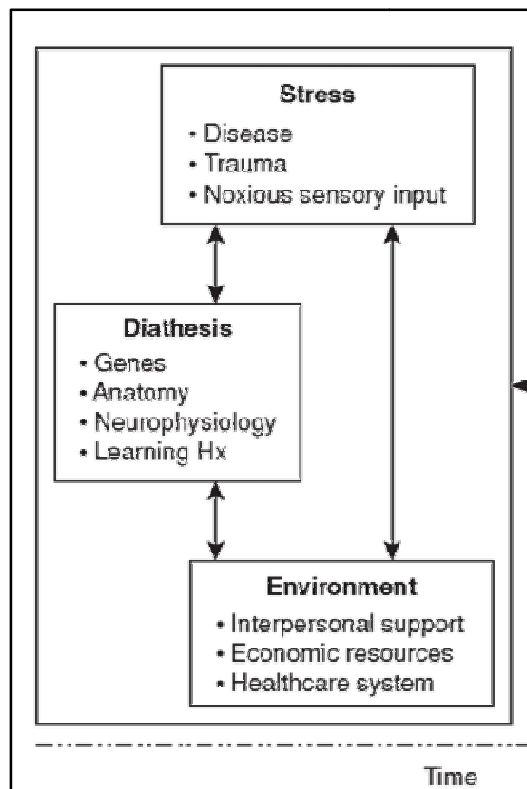
Cognitive Behavioural Case Formulation



Dialectical Behavioural Formulation – Biosocial Theory



Diathesis Stress Model



Biopsychosocial Model - Engel

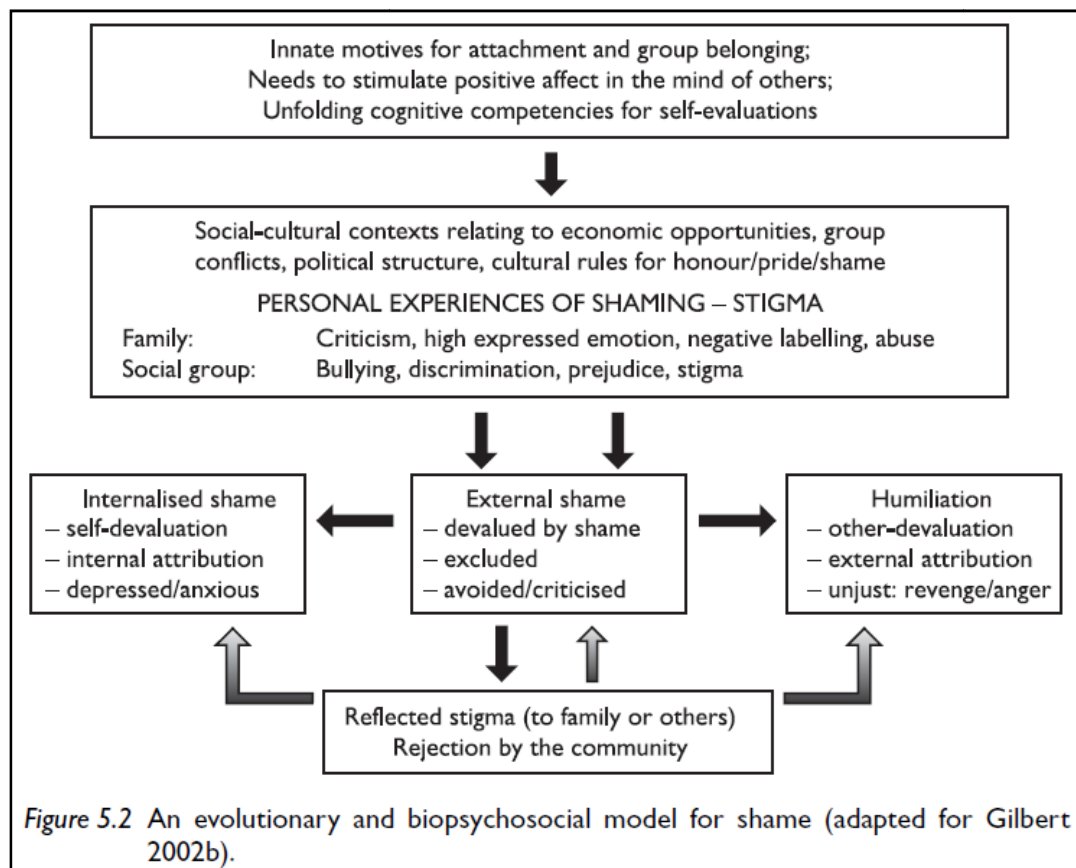
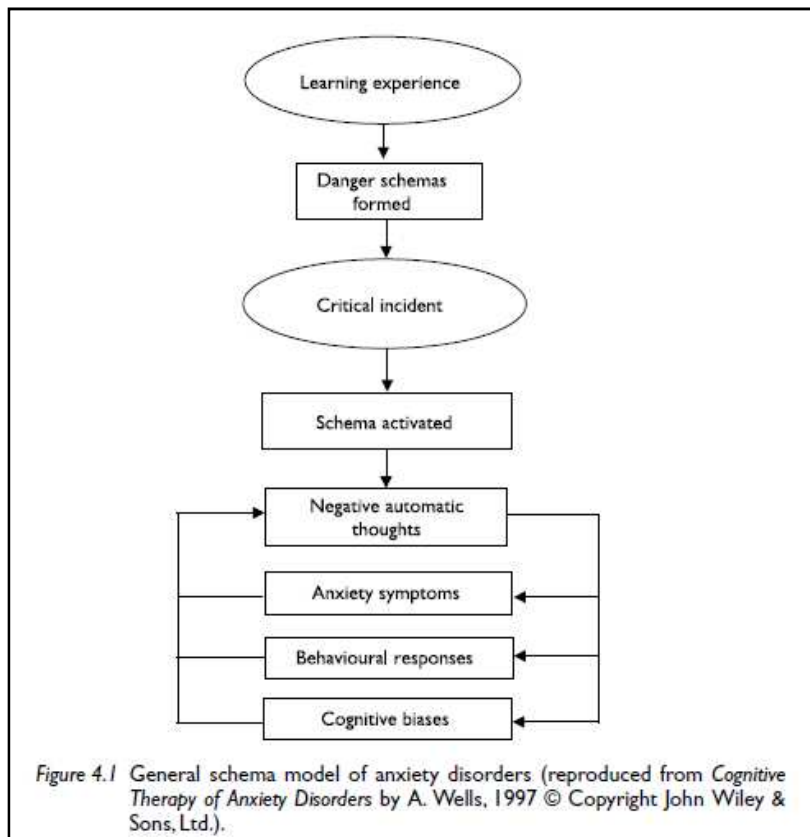


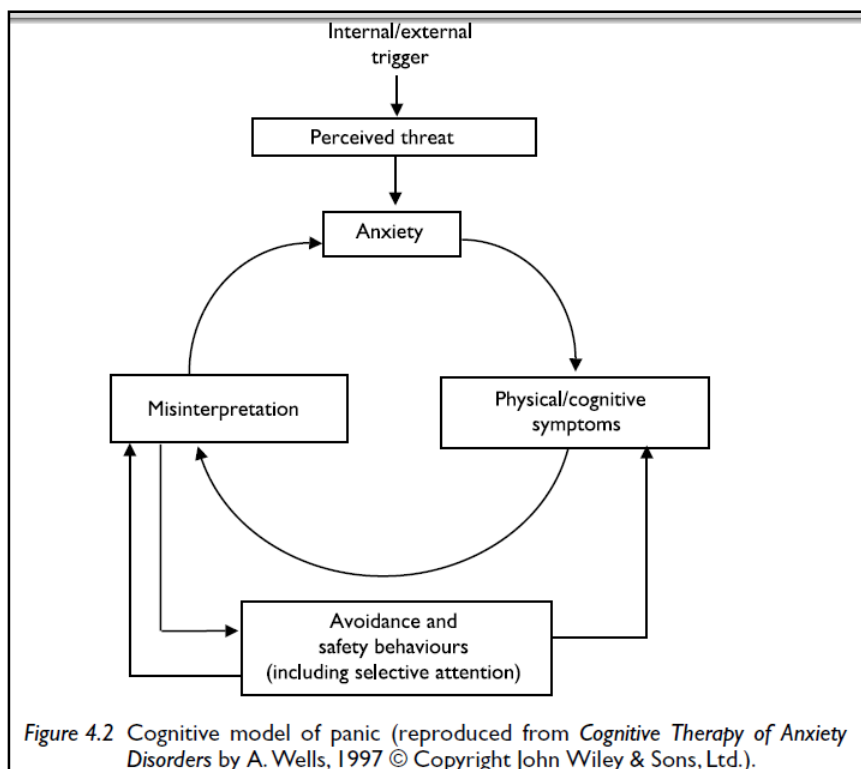
Figure 5.2 An evolutionary and biopsychosocial model for shame (adapted for Gilbert 2002b).

Formulations conceptualizing Specific Disorders

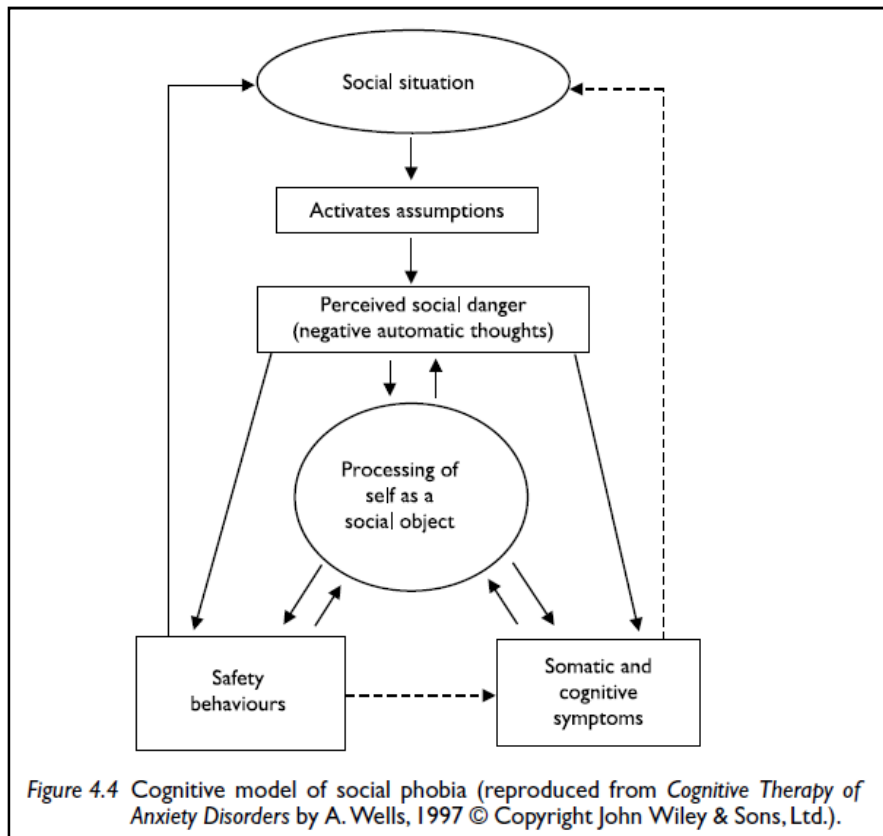
1. Anxiety Disorders



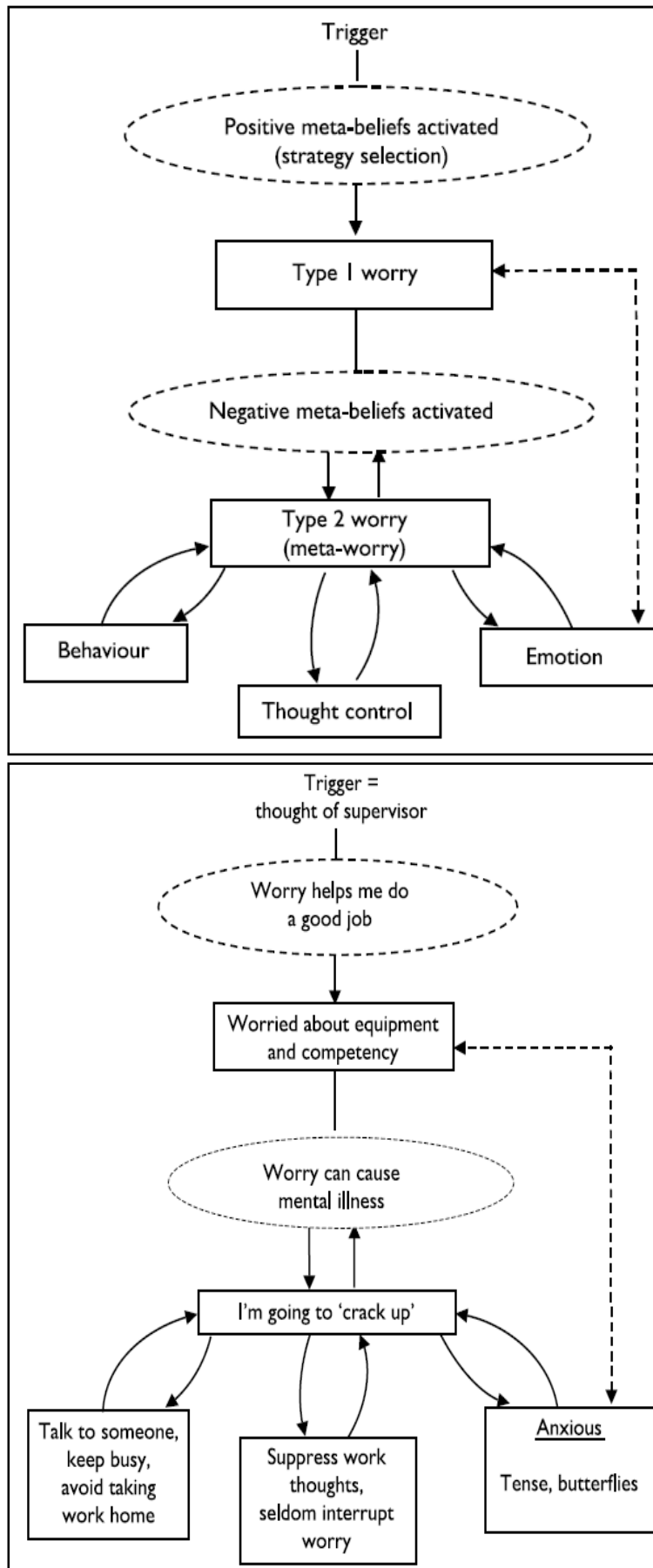
2. Panic Disorder



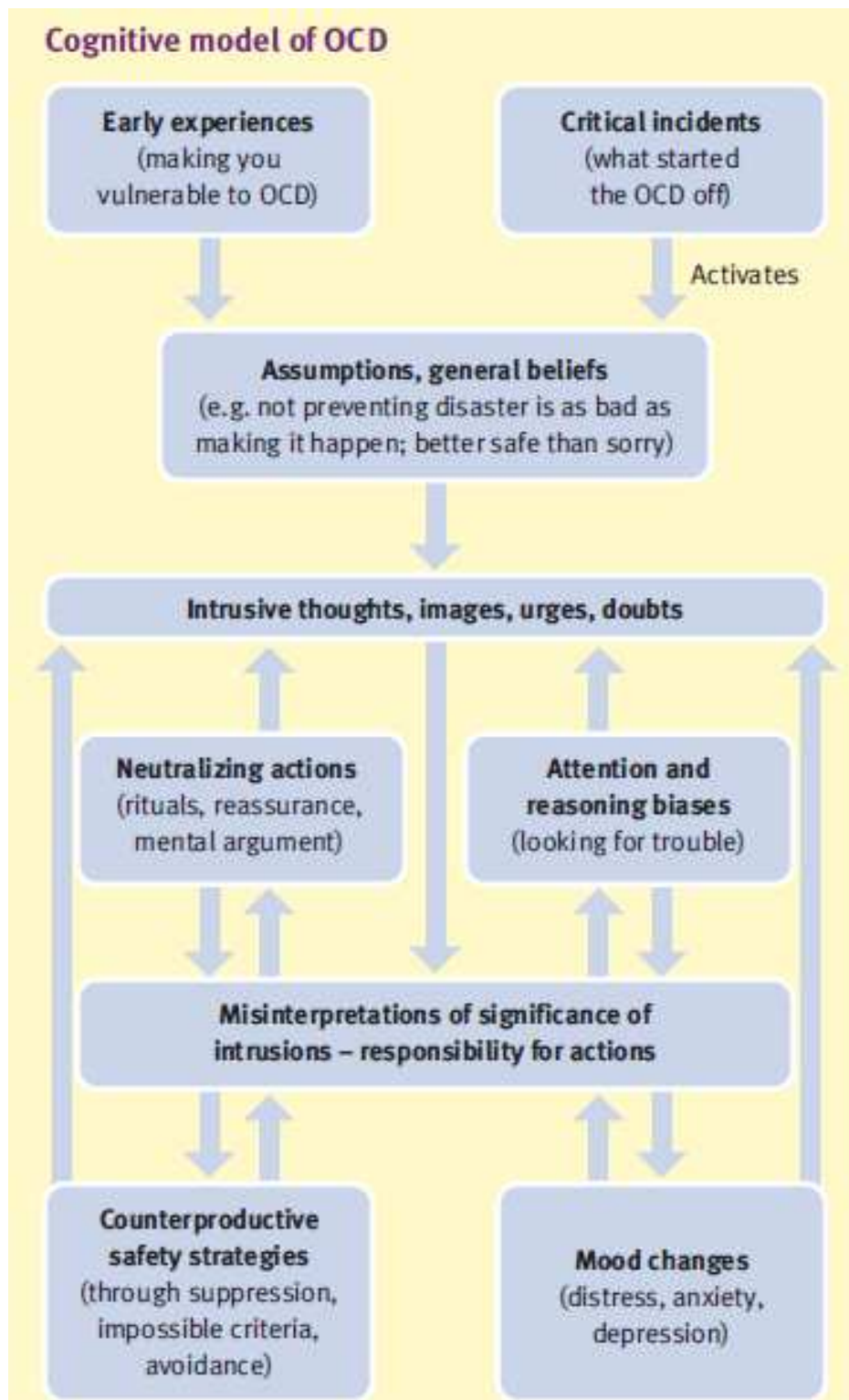
3. Social Phobia



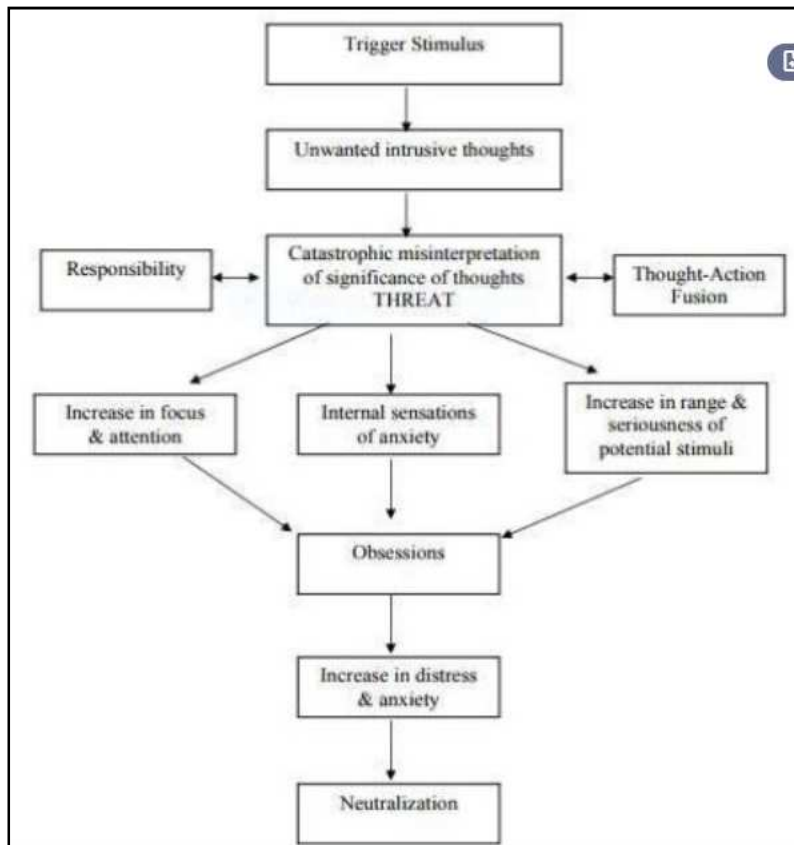
4. Generalized Anxiety Disorder



5. Obsessive Compulsive Disorder
Salkovskis' Model

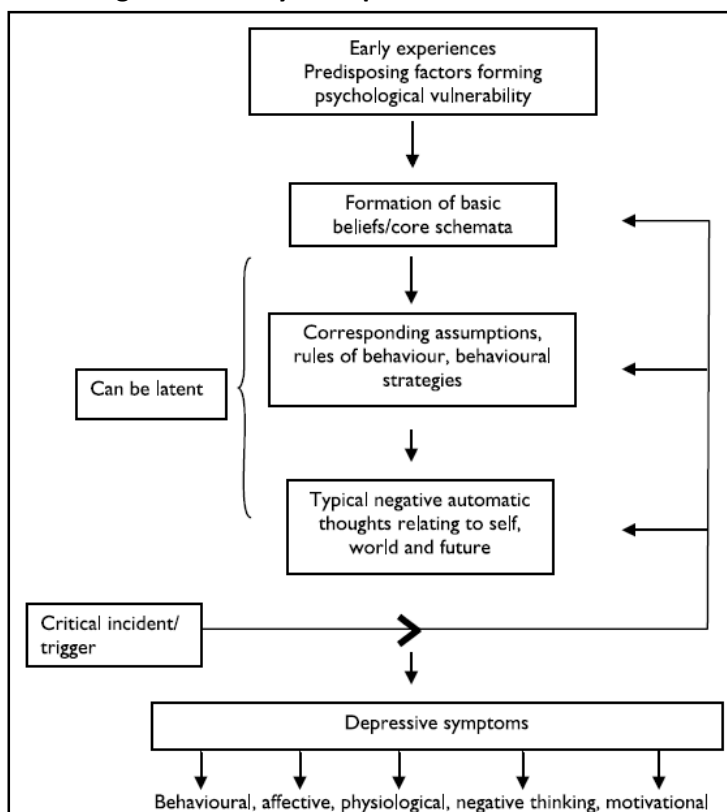


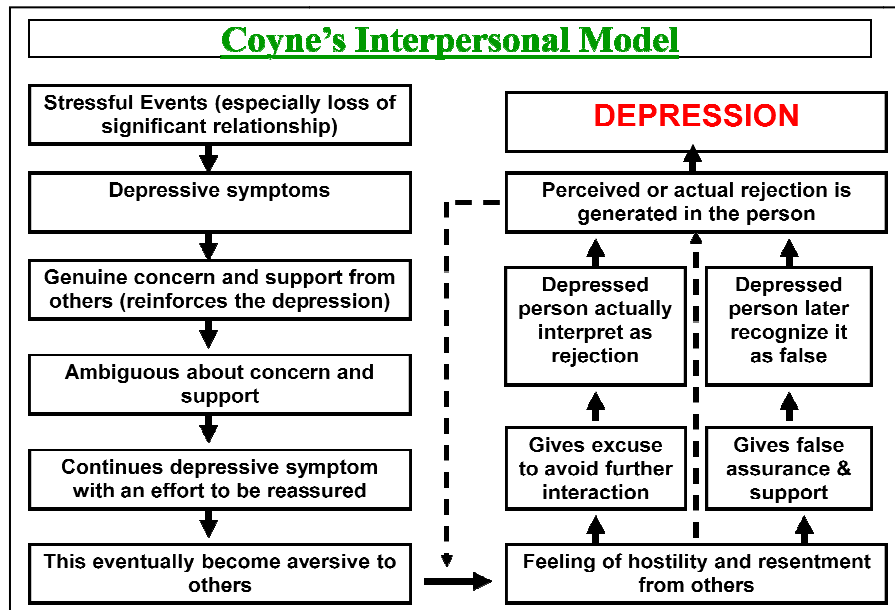
Rachman's Model of OCD



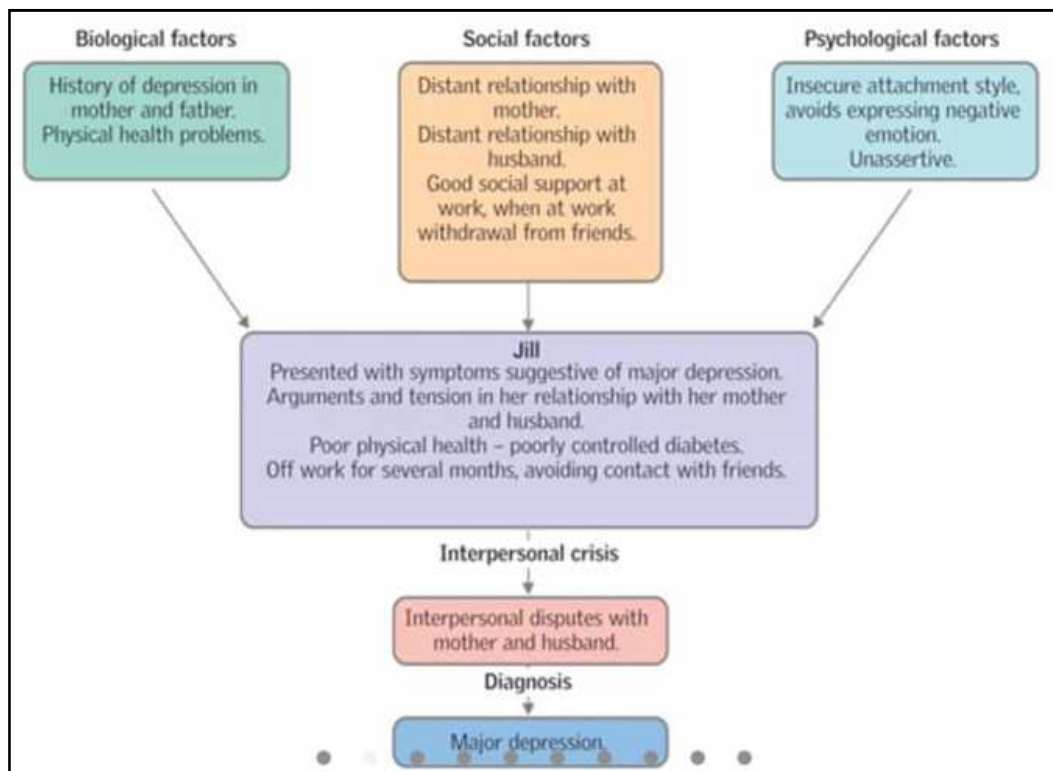
6. Depression

Beck's Cognitive Theory of Depression

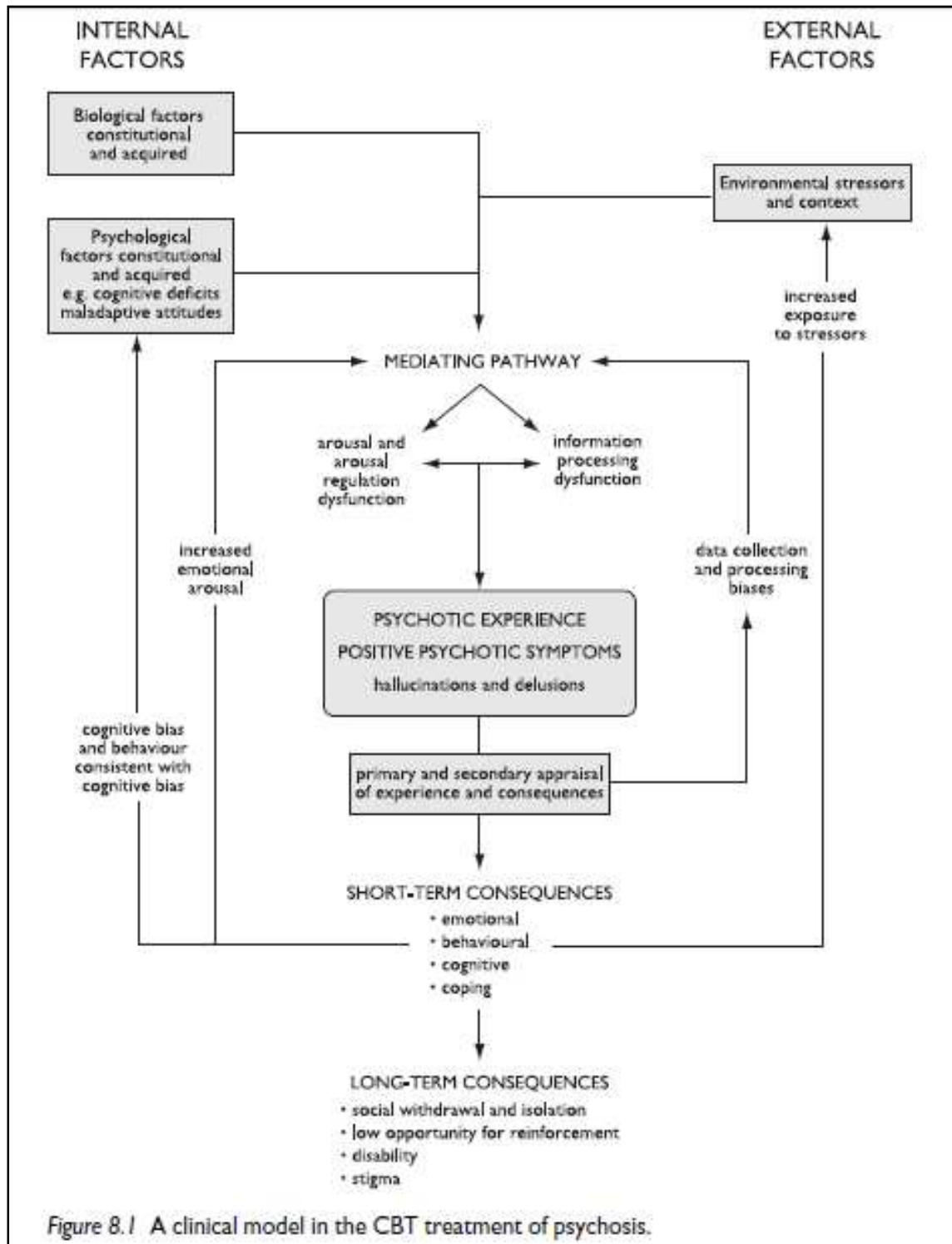




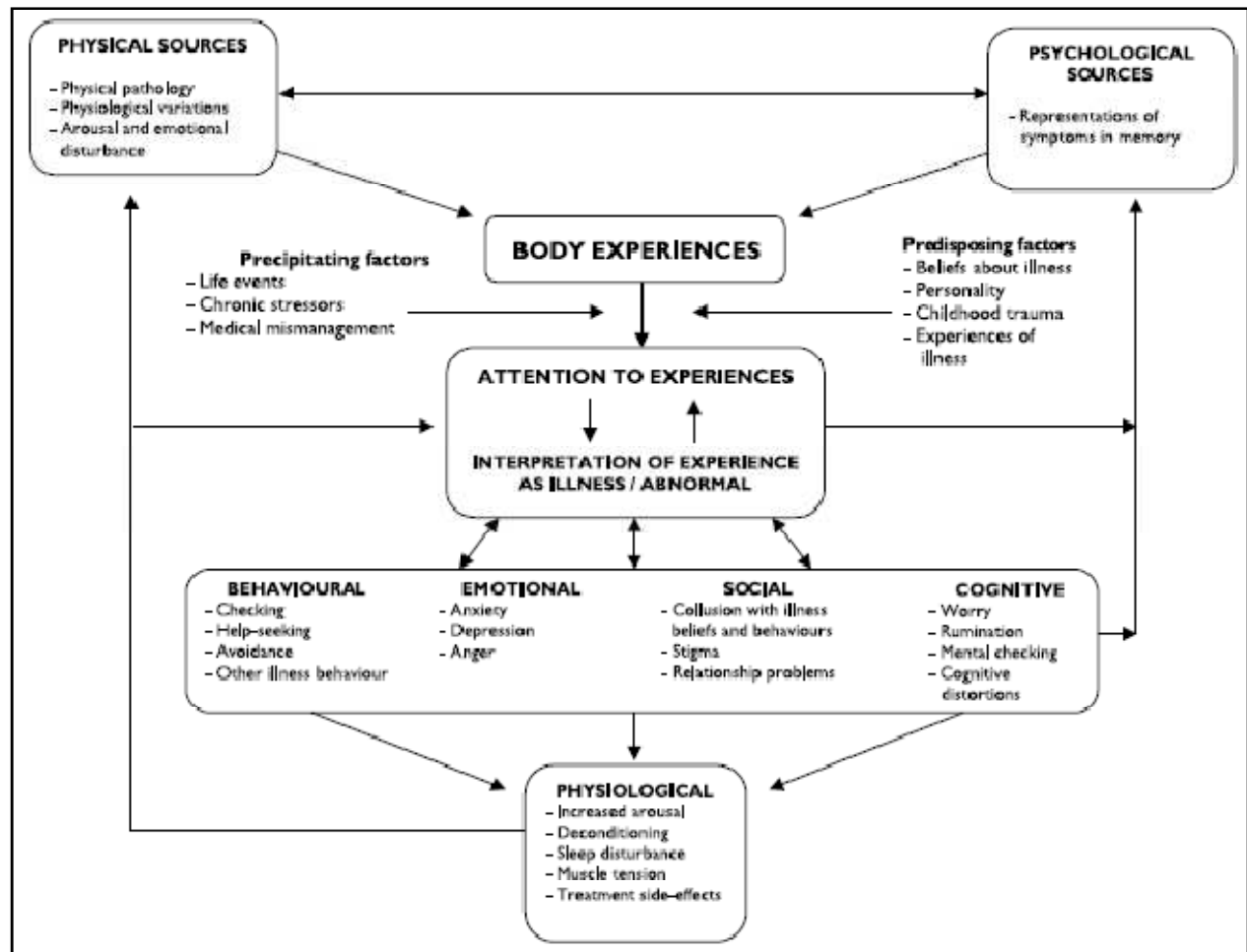
Biopsychosocial Model of Depression



7. Psychosis

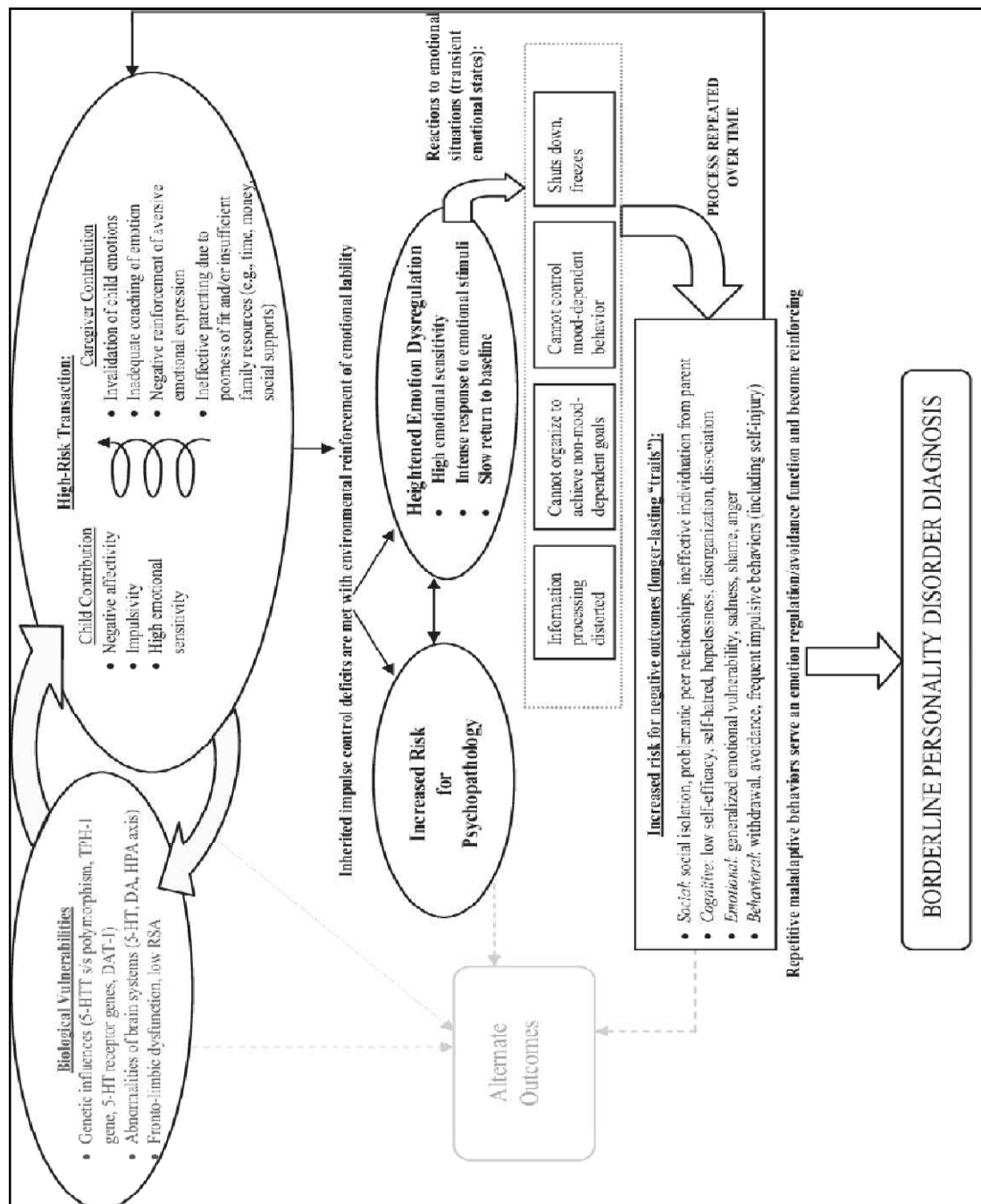


8. Medically Unexplained Symptoms



9. Personality Disorder

Biosocial Developmental Model - Linehan



Schema Therapy based Formulation for Personality Disorder

Table 1: Domains and Schemas

Disconnection & Rejection Abusive, traumatic childhoods; unstable family life; rejection and humiliation; feel different and lacking in some way; long periods of insecurity and inconsistent parenting	Mistrust/Abuse Abandonment/Instability Emotional Deprivation Defectiveness/Shame Social Isolation/Alienation
Impaired Autonomy & Performance Often over protected and controlled as children, or neglected and ignored, left alone with no interest shown in their lives; continually undermined and made to feel incompetent, or encouraged to be dependent on others	Dependence/Incompetence Vulnerability to Harm Enmeshment Failure
Impaired Limits Internal sense of control not developed; difficulty respecting the rights of others; families very un-boundaried; children did not have rules	Entitlement Insufficient Self Control/Self Discipline
Other Directedness Experienced conditional love; family overly concerned with appearances; parents focused on their own needs	Subjugation Self-Sacrifice Approval Seeking/Recognition Seeking
Over-vigilance and Inhibition Strict parental control to gain compliance; ever watchful - waiting for bad things to happen; frightened of severe punishments for expression of feelings	Negativity/Pessimism Emotional Inhibition Unrelenting standards/ Hypercriticalness Punitiveness

Cognitive Formulation of Personality Disorder

